



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ياسمين نصري بخيت موسي

المحافظة : القليوبية الحي / المدينة : شبرا الخيمة ثان العنوان : //

رقم البطاقة : 27904170101829 رقم التليفون : 01289464419 تاريخ تسجيل الإستمارة : 24-09-2026

العمر (تاريخ الميلاد) : 17-04-1979 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ياسمين نصري بخيت موسى National ID: 27904170101829	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ياسمين
نصرى بخيت موسى
اش اسماعيل عبدالفتاح-عزيز بدران
شبرا الخيمة ثان - القليوبية

١٩٧٩/٠٢ ٢٧٩٠٤١٧٠١٠١٨٢٩

IU0865474

١٢٨٩٤٦٤٤١٩
١٢٠٧٣٥٩٤٣٣

٢٠٢١/٠٢ ٢٧٩٠٤١٧٠١٠١٨٢٩

أنثى مسيحية متزوجة
سعيد بخيت تاوضروس عبدالملاك
البطاقة سارية حتى ٢٠٢٨/٠٢/٢٢ ١١



Name	YASMIN NASRY
Date	7-7-2026

MRI of Both Breasts with contrast revealed: -

Technique: -Variable pulse sequences were taken.

Findings: -

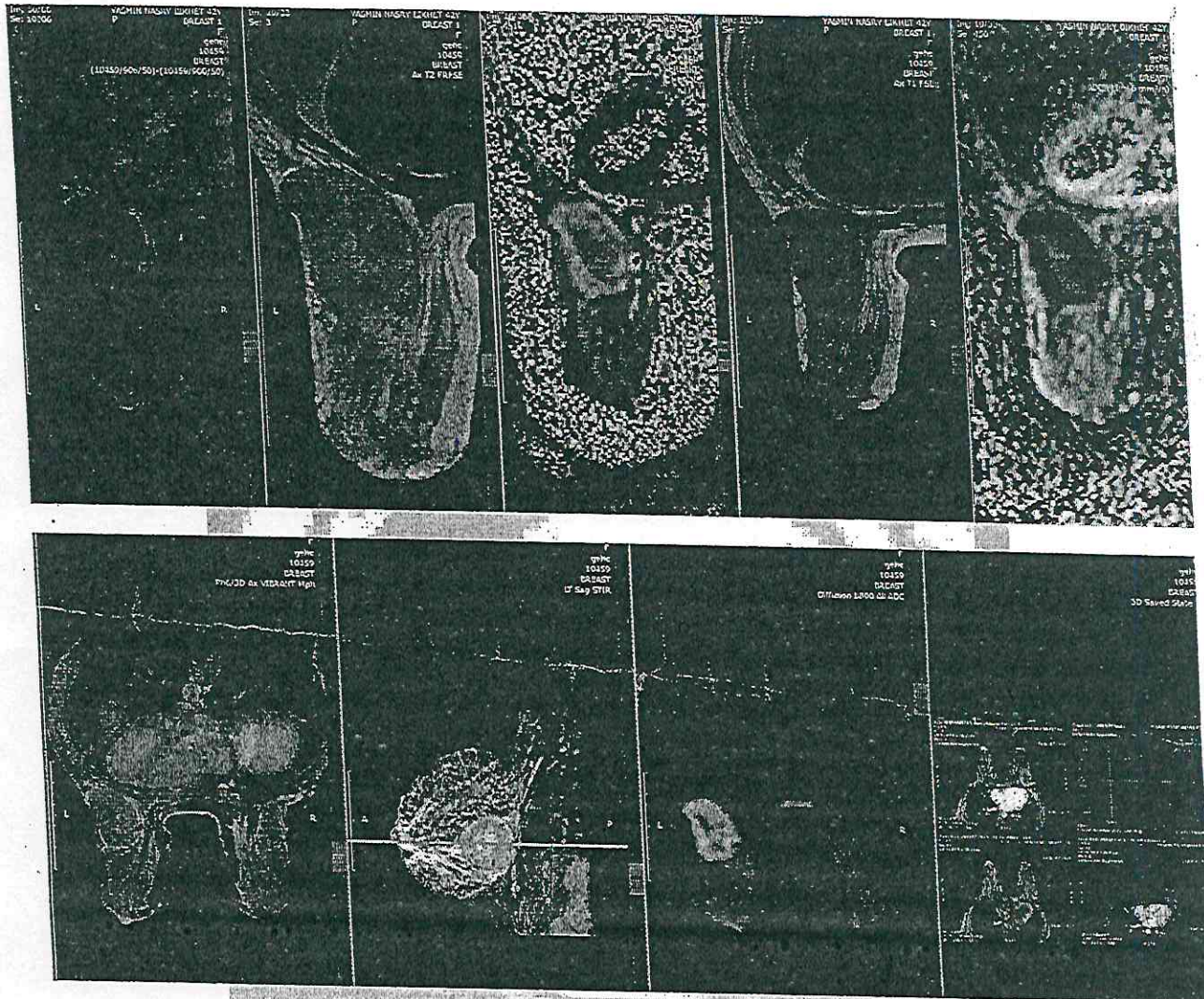
- Both breasts shows normal appearance of their glandular and fatty components (fibro-glandular tissue)
- Both breasts shows dense glandular and fatty components (type D)
- Asymmetric moderate Moderate BPE 51-75% Background Parenchymal Enhancement(BPE)
- **Left Breast**
Mass seen at upper outer quadrant measure 58x41 mm. partially circumscribed Irregular border, lobulated, abutting related pectoralis muscle posteriorly .low signal T1 intermediate heterogeneous T2 signal with central high degeneration signal .restricted diffusion with central no restriction
Internal heterogeneous enhancement with persistent curve, Peripheral rim enhancement
- **Right breast**
no definite focal lesions or abnormal pattern of enhancement.
No masses appear worrisome for malignancy based upon MRI criteria
- Normal skin thickness no skin enhancement.
- Intact anterior chest wall
- imaged parts of both axillae show bilateral benign looking nonspecific lymphadenopathy
(Pre- and post-contrast images demonstrate circumscribed, reniform intense homogenous cortex enhancement of a axillary lymph node that has a benign appearance. demonstrates Central fatty hilum. Preserved cortical hilar ratio)

Opinion:

1. Pathology report show atypical papillary lesion
2. The MRI appearance of left breast mass raises concern that the lesion may harbor malignancy Score BIRAD 4C
3. The standard recommendation is:
 - a-Complete surgical excision (excisional biopsy/lumpectomy) because atypical papillary lesion have a significant risk of upgrade to DCIS or invasive carcinoma on final pathology.
 - b-Imaging-pathology correlation in a multidisciplinary breast meeting.



Name	YASMIN NASRY
Date	7-7-2026



Pathology Report

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026. Cold ischemia time is 20 seconds.

- Multiple fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.5 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

- Histologic type: Extensively necrotized invasive duct carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2).

- Histologic Grade: 2 (intermediate)
- DCIS: Absent
- Microcalcification: Absent
- Angiolymphatic Invasion: Absent
- Perineural invasion: Absent

* IPT (All controls showed appropriate reactivity):

- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67%: 60%.

Left breast mass, Tru-cut biopsy:

- Extensively necrotized invasive duct carcinoma, Grade 3. IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 60%.



Patient Name	باسمين نصري بنيت	Case No.	358673
Sex/Age	Female 47 Y	Patient No.	25986
Registered Date	17/8/2026 01:55:28 PM	Referred By Dr.	Interventional Radiology
Validation Date	07/9/2026 01:54:04 PM	Branch	Gustave Roussy
Reporting Date	24/9/2026 09:10:48 AM	Pathology No.	2605/26

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

*Signature***Dr. Walaa Ibrahim Ghoneim, MD.**

Patient Name	ياسمين نصرى بنت	Case No.	358673
Sex/Age	Female 47 Y	Patient No.	25986
Registered Date	17/8/2026 01:55:28 PM	Referred By Dr.	Interventional Radiology
Validation Date	07/9/2026 01:54:04 PM	Branch	Gustave Roussy
Reporting Date	24/9/2026 09:10:48 AM	Pathology No.	2605/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026. Cold ischemia time is 20 seconds.

Gross Examination:

- Multiple fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.5 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Extensively necrotized invasive duct carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2).
- DCIS: Absent.
- Microcalcification: Absent.
- Angiolinvasion: Absent.
- Perineural invasion: Absent.

Results:

* PT (All controls showed appropriate reactivity):

- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67%: 60%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Extensively necrotized invasive duct carcinoma, Grade 3, IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 60%.



Patient Name	باسمى نصرى بنت	Case No.	358673
Sex/Age	Female 47 Y	Patient No.	25986
Registered Date	17/8/2026 01:55:28 PM	Referred By Dr.	Interventional Radiology
Validation Date	07/9/2026 01:54:04 PM	Branch	Gustave Roussy
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, and 5, between 67%-100% of the cells staining). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim, MD.
